

Locum Tenens Rate Guide 2026

Real compensation data — hourly rates by specialty, agency markups, and total cost of coverage

- ✓ Hourly and daily rates for 20+ physician specialties
- ✓ Agency markup breakdown: what you actually pay vs. what the provider earns
- ✓ Hidden costs beyond the bill rate (housing, travel, malpractice tail)
- ✓ How to negotiate better rates and reduce total locum spend

How Locum Tenens Billing Works

Locum tenens billing is an all-inclusive package, not a simple hourly rate. The bill rate your facility pays covers the clinician's compensation, agency operational costs, housing, travel, and malpractice insurance. Understanding each component is essential to evaluating whether you're getting a fair deal.

Bill Rate Structure Overview

Component	Description	Typical % of Bill Rate
Clinician compensation	Direct pay to the physician or APP	55–70%
Agency markup	Recruitment, credentialing, compliance, support, profit	25–40%
Housing & travel	Furnished housing, flights/rental car, per diem	Included or billed separately
Malpractice insurance	Occurrence-based coverage during assignment	Included in agency markup

Quick Budget Reference — Physician Bill Rates (What Your Facility Pays the Agency)

- Primary Care (FM/IM): \$175–\$250/hour
- Hospitalist: \$175–\$275/hour
- Emergency Medicine: \$250–\$400/hour
- Anesthesiology: \$350–\$500/hour
- General Surgery: \$275–\$400/hour
- Psychiatry: \$200–\$350/hour
- Advanced Practice Provider (NP/PA): \$90–\$150/hour

Rates by Specialty (Physician)

2026 Locum Tenens Hourly Rates by Specialty — What Facilities Pay

The following rates represent the all-in bill rate (what facilities pay the agency), not the clinician's take-home pay. Clinician compensation is typically 55–70% of the bill rate after agency markup.

Specialty	Hourly Bill Rate	Daily Rate (10-hr)	Weekly Rate (5 days)	Key Notes
Family Medicine	\$150–\$220	\$1,500–\$2,200	\$7,500–\$11,000	Highest volume specialty; most availability
Internal Medicine	\$160–\$230	\$1,600–\$2,300	\$8,000–\$11,500	Strong demand in rural facilities
Hospitalist	\$175–\$275	\$1,750–\$2,750	\$8,750–\$13,750	7-on/7-off schedule common
Emergency Medicine	\$250–\$400	\$2,500–\$4,000	\$12,500–\$20,000	Wide range by volume and acuity
Psychiatry	\$200–\$350	\$2,000–\$3,500	\$10,000–\$17,500	Severe shortage; telepsychiatry often available
Anesthesiology	\$300–\$500	\$3,000–\$5,000	\$15,000–\$25,000	Highest-cost specialty; CRNAs are lower
General Surgery	\$275–\$400	\$2,750–\$4,000	\$13,750–\$20,000	Call coverage requirements affect pricing
Orthopedic Surgery	\$300–\$450	\$3,000–\$4,500	\$15,000–\$22,500	Sub-specialty premiums for spine/joints
OB/GYN	\$200–\$325	\$2,000–\$3,250	\$10,000–\$16,250	Call coverage requirements increase cost
Cardiology	\$250–\$400	\$2,500–\$4,000	\$12,500–\$20,000	Interventional commands premium
Radiology	\$200–\$350	\$2,000–\$3,500	\$10,000–\$17,500	Teleradiology locums often cheaper
Pulmonology / Critical Care	\$225–\$375	\$2,250–\$3,750	\$11,250–\$18,750	ICU coverage premiums

Specialty	Hourly Bill Rate	Daily Rate (10-hr)	Weekly Rate (5 days)	Key Notes
Dermatology	\$175-\$300	\$1,750-\$3,000	\$8,750-\$15,000	High patient volume makes ROI strong
Neurology	\$225-\$350	\$2,250-\$3,500	\$11,250-\$17,500	Telestroke opportunities
Urology	\$250-\$375	\$2,500-\$3,750	\$12,500-\$18,750	Procedural volume key
Medical Oncology	\$375-\$500	\$3,750-\$5,000	\$18,750-\$25,000	Highest-compensated medical subspecialty
Pediatrics	\$150-\$225	\$1,500-\$2,250	\$7,500-\$11,250	Lower reimbursement limits facility budget
Pathology	\$175-\$275	\$1,750-\$2,750	\$8,750-\$13,750	Often remote/telepathology

Advanced Practice Provider (APP) Rates

Provider Type	Hourly Bill Rate	Daily Rate (10-hr)	Notes
Nurse Practitioner (NP)	\$90-\$150	\$900-\$1,500	Primary care, urgent care, specialty
Physician Assistant (PA)	\$90-\$150	\$900-\$1,500	Surgery assist commands premium
CRNA	\$175-\$275	\$1,750-\$2,750	Significant savings vs. anesthesiologist

Agency Markup Breakdown

What the Agency Keeps — Understanding the Markup

Agency markup funds recruitment, credentialing, compliance, 24/7 support, and the agency's profit margin. Industry markups range from 25% to 60% above the clinician's compensation, with 30–50% being the most common range.

Markup Breakdown Example

Component	Amount	% of Bill Rate
Clinician hourly pay	\$120–\$140	60–70%
Agency operational costs	\$20–\$30	10–15%
Housing & travel amortized	\$15–\$25	8–12%
Malpractice insurance	\$5–\$10	3–5%
Agency profit	\$10–\$20	5–10%
Total bill rate	\$200	100%

Factors That Increase Markup

Factor	Impact on Markup
Urgent/ASAP start (< 2 weeks)	+10–25% above standard rates
Rural or undesirable location	+10–20% location premium
Holiday or peak season coverage	+15–30% premium
Hard-to-fill specialty	Higher base rate + higher markup
Short-term assignment (< 4 weeks)	+10–15% vs. longer assignments
Call coverage requirements	+\$1,000–\$3,000/week premium

Key Insight

The agency markup is NOT pure profit. A significant portion covers credentialing (30–90 days of work per provider), malpractice insurance, housing arrangements, travel booking, and 24/7 support. However, markups above 50% warrant scrutiny — ask the agency for a rate breakdown.

Rates by Region & Facility Type

Region / Setting	Bill Rate Premium vs. National Avg	Key Notes
Rural Critical Access Hospitals	+15-30%	Provider shortage premium; housing limited
Urban Academic Medical Centers	-5-10% (standard/below)	Prestige attracts providers; more applicants
Community Hospitals (suburban)	Standard	Largest market segment
VA / Federal Facilities	Fixed GSA rates	Government rates; less negotiable
FQHCs / Community Health Centers	-10-20%	Lower budgets; may offset with J-1/ H-1B waiver
Northeast (NYC, Boston)	+10-20%	High cost of living; expensive housing
California	+15-25%	Highest state; licensing delays add cost
Texas / Southeast	Standard to -5%	Growing markets; competitive supply
Midwest / Plains	+5-15%	Rural premium for smaller markets
Pacific Northwest	+5-10%	Moderate premium; desirable locations

California Licensing Warning

California medical license processing takes 60-90+ days. If your locum needs a California license, plan 3+ months ahead or pay a premium for providers who already hold one. Agencies charge more for California-licensed physicians because of this bottleneck.

Hidden Costs & Add-Ons

The hourly bill rate is not the total cost. Budget an additional 10–25% above the bill rate for costs that are billed separately or not immediately apparent.

Cost Item	Typical Range	When It Applies
Housing (if not included in bill rate)	\$2,500–\$6,000/month	Some agencies bill housing separately
Travel (flights, rental car)	\$500–\$2,000 per assignment	Each time a new locum starts or returns after time off
Malpractice tail coverage	\$5,000–\$25,000+	If policy is claims-made (not occurrence); due when locum leaves
Credentialing costs	\$500–\$2,000	Some agencies pass this through; most absorb it
Orientation/onboarding (non-billable days)	1–3 days at full rate	Provider is paid but not generating revenue
Call coverage premium	\$1,000–\$3,000/week	Beyond standard shift hours
Overtime (> 40 hrs/week or > 12 hrs/day)	1.5x bill rate	Some contracts specify OT thresholds
Extension/holdover premium	+5–15%	Extending beyond original assignment dates
Cancellation fee (< 30 days notice)	1–4 weeks of bill rate	Canceling an assignment before start date
Conversion fee (hire permanently)	\$15,000–\$50,000	Locum-to-perm conversion; often 15–25% of annual salary
Credentialing rush fee	\$500–\$2,500	Expedited credentialing in under 2 weeks
License procurement	\$1,000–\$3,000	Agency assists with new state license for locum

Always ask whether housing and travel are included in the quoted bill rate or billed separately. An agency quoting \$200/hr with housing included is cheaper than one quoting \$185/hr plus \$4,000/month housing. Calculate the true all-in cost.

How to Negotiate Better Rates

Numbered list

- 1 Extend assignment duration.** Longer assignments (13+ weeks) command lower rates than short-term coverage (< 4 weeks). Agencies amortize recruitment and travel costs over more billable hours.
- 2 Plan ahead — avoid urgent fills.** Giving agencies 30–60 days lead time (vs. "need someone Monday") can reduce rates by 10–25%. Urgent fills carry a premium.
- 3 Use multiple agencies competitively.** Get quotes from 3+ agencies for the same assignment. Rate variation between agencies for identical positions can be 15–30%.
- 4 Negotiate the conversion fee upfront.** If there's any chance you'll want to hire the locum permanently, negotiate the conversion fee before the assignment starts. Standard is 15–25% of first-year salary.
- 5 Consolidate locum spend with fewer agencies.** Volume commitments to 1–2 preferred agencies yield better rates than spreading across many. Negotiate a master services agreement (MSA).
- 6 Consider APPs where appropriate.** A nurse practitioner or PA at \$90–\$150/hour can cover many roles that don't require a physician, saving 40–60% on bill rates.
- 7 Offer desirable scheduling.** 7-on/7-off, no call, reasonable patient loads, and good EMR systems attract more applicants, increasing competition and lowering rates.
- 8 Negotiate housing directly.** Some facilities save by providing housing directly (hospital-owned apartments, negotiated corporate housing) rather than paying the agency's housing markup.
- 9 Request rate transparency.** Ask the agency to disclose the clinician's pay rate and the markup percentage. Not all agencies will comply, but those that do demonstrate integrity.
- 10 Lock rates for the fiscal year.** Negotiate rate caps or maximum annual increases (e.g., CPI + 2%) in your MSA to prevent mid-year rate hikes during shortages.

Budget Planning Worksheet

Use this worksheet to estimate total locum tenens costs per assignment. Print and fill in for each coverage need.

Line Item	Estimated Cost	Actual Cost
Bill rate (hourly × hours/week × weeks)	\$ _____	\$ _____
Housing (if billed separately)	\$ _____	\$ _____
Travel (per trip × number of trips)	\$ _____	\$ _____
Call coverage premium (per week)	\$ _____	\$ _____
Orientation/onboarding (non-productive days)	\$ _____	\$ _____
Credentialing costs (if pass-through)	\$ _____	\$ _____
License procurement (if needed)	\$ _____	\$ _____
Overtime estimate	\$ _____	\$ _____
Malpractice tail (if claims-made)	\$ _____	\$ _____
Potential conversion fee	\$ _____	\$ _____
15% contingency buffer	\$ _____	\$ _____
TOTAL ESTIMATED ASSIGNMENT COST	\$ _____	\$ _____

Complete this worksheet before requesting quotes. Knowing your full scope (hours, call, duration, start date urgency) helps you get accurate all-in pricing and compare agencies apples-to-apples.

CTA box

Want a quick sanity check on your numbers? Email this worksheet to contact@locumtrust.com or browse verified locum tenens agencies at locumtrust.com

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